PORTFOLIO PROJECT 20 · 2026-2027

Healthcare Consulting Problem-Framing & Implementation Readiness Intelligence™

A student-developed case study that converts broad healthcare consulting into a practical method for defining operational problems, separating symptoms from root causes, prioritizing workstreams, designing controls, testing solutions, measuring results, and protecting the patient experience.

> Created by Kori Pickle, BSHA Candidate, University of Phoenix. Educational, simulated, and no PHI. No real patient, employer, payer, consulting-client, claims, EHR, portal, or organizational outcome data were used.

HEALTHCARE OPERATIONS INTELLIGENCE ENGINE™

Diagnose
Design
Implement
Sustain

CORE QUESTION

Where did the workflow first lose control?

Kori Pickle · BSHA Candidate

EXECUTIVE BREAKDOWN

The post describes a growing industry. The real value is disciplined problem solving.

What the post gets right

Healthcare consulting can span strategy, finance, operations, technology, revenue cycle, quality, workforce, compliance support, and transformation. Organizations may seek outside expertise when a problem crosses departments or requires specialized implementation capability.

What requires caution

Market projections are commercial forecasts, not guarantees. One 2025 vendor report projected the market from $32.2 billion in 2025 to $52.0 billion by 2030 at a 10.1% CAGR, but definitions and methods vary across firms.

Consulting is not a logo list

The practical work is defining the decision, collecting evidence, locating root causes, designing a future state, testing it, supporting adoption, and proving whether the change worked.

Honest student positioning

This portfolio demonstrates consultant-style reasoning and implementation readiness without claiming client work, executive authority, or professional consulting experience.

> A consulting engagement is a controlled path from an unclear problem to measurable, sustainable change.

SERVICE DOMAIN MAP

What healthcare consulting teams may actually work on.

Operations

Patient flow, access, scheduling, staffing, capacity, standard work, handoffs, productivity, and service recovery.

Revenue cycle

Eligibility, authorization, documentation, charge capture, denials, payment posting, payer workflow, and patient balances.

Technology

EHR optimization, interoperability, implementation, automation, data governance, cybersecurity, and workflow adoption.

Quality & value

Measurement, variation, patient experience, care coordination, quality programs, and value-based care design.

Strategy & finance

Growth planning, service-line analysis, business cases, operating models, partnerships, and investment priorities.

Change & implementation

Governance, requirements, testing, training, adoption, issue management, transition, and sustainment.

> The deliverable is not the slide deck. The deliverable is a change with evidence, ownership, adoption, and measurable closure.

CONSULTANT-STYLE TOOLKIT

Seven tools that turn a broad concern into an actionable engagement.

Problem statement

Define the observed issue, affected population, operational impact, timeframe, boundaries, and evidence without assuming the cause.

Current-state map

Show steps, systems, owners, handoffs, wait states, exceptions, rework loops, and patient-facing communication.

Issue tree

Separate potential causes across people, process, technology, policy, data, demand, capacity, and external dependencies.

Stakeholder map

Identify decision owners, users, subject-matter experts, patients, technology partners, and escalation authorities.

Evidence matrix

Record the source, date, confidence, limitation, owner, and additional validation required for each finding.

Prioritization model

Rank work by patient impact, operational risk, financial exposure, effort, dependency, feasibility, and urgency.

Implementation charter

Define scope, deliverables, acceptance criteria, governance, risks, testing, training, metrics, transition, and sustainment.

Responsible AI boundary

AI may support drafting, summarization, categorization, or pattern review, but recommendations need approved sources, human review, traceability, and risk controls.

SIMULATED NO-PHI CASE

A health system asks for "a digital transformation." The problem is actually operational.

A simulated multi-site outpatient organization reports scheduling delays, authorization aging, avoidable denials, duplicate work, conflicting patient instructions, and unreliable dashboards. Leadership initially frames the solution as purchasing a new platform.

Patient delays	Leaders assume the scheduling system is slow, but incomplete intake, authorization, routing, rework, and ownership may be driving the delay.
Authorization aging	Leaders assume more staff are needed, but the backlog may be concentrated by service, payer, missing document, handoff, or deadline.
Denial growth	Leaders assume coding is the problem, but the first loss of readiness may be registration, eligibility, authorization, documentation, routing, or payer-rule translation.
Dashboard conflict	Technology is blamed even though teams may be using different status definitions, timestamps, sources, and closure rules.
First loss of control	The organization did not share one definition of readiness, one exception taxonomy, or one accountable owner across scheduling, authorization, documentation, and claim preparation.

> The first consulting risk was solution-first thinking: selecting technology before defining the workflow problem, evidence, and acceptance criteria.

ENGAGEMENT LIFECYCLE

Eight stages from unclear concern to sustainable ownership.

01	02	03	04	05	06	07	08
FRAME	DISCOVER	DIAGNOSE	PRIORITIZE	DESIGN	TEST	IMPLEMENT	SUSTAIN

Frame & discover

Define scope and decisions, then collect workflows, policies, interviews, system evidence, metrics, patient feedback, and limitations.

Diagnose & prioritize

Separate symptoms from causes, validate findings, quantify impact, and rank work by risk, impact, feasibility, effort, and dependency.

Design & test

Create future-state workflows, ownership, requirements, controls, measures, communications, and synthetic acceptance tests.

Implement & sustain

Train users, manage issues, monitor adoption, transfer ownership, audit exceptions, and close the feedback loop.

Healthcare Consulting Implementation Readiness Matrix™

Workstream	Required design	Evidence of readiness	Escalation trigger
Problem governance	Sponsor, decision owner, scope, steering cadence, escalation path	Charter, decision log, named issue owners	Scope conflict, delayed decision, unresolved ownership
Workflow	Current and future states, gates, exception paths, dependency resets	Approved maps and role-specific standard work	Work advances with missing evidence or unclear handoff
Data	Definitions, source hierarchy, timestamps, quality rules, lineage, access	Data dictionary, reconciliation, quality thresholds	Dashboards disagree or sources are stale/incomplete
Technology	Requirements, interfaces, configuration, security, fallback, monitoring	Traceable requirements and passed test cases	Critical defect, silent failure, unapproved data use
People & change	Stakeholder analysis, communication, training, support, adoption measures	Role-based training and competency evidence	Low adoption, workarounds, resistance, or safety concern
Patient experience	Clear instructions, accessibility, language support, financial transparency	Approved content, test feedback, complaint routing	Conflicting message or inaccessible pathway
Responsible AI	Approved purpose, limited data, human review, traceability, fallback	Use-case register, reviewer, audit record	Untraceable output, bias concern, low confidence, patient impact
Cyber resilience	Asset inventory, access control, downtime, vendor risk, recovery	Tested response and reconciliation procedures	Unmanaged asset, outage, suspicious activity, unreconciled work

> Implementation readiness exists when process, data, technology, people, patient safeguards, governance, and measurement are ready together.

SYNTHETIC UAT

Test the recommendation before a real patient absorbs the defect.

Coverage or service change

Dependent payer, network, authorization, estimate, and routing statuses reset while preserving the audit trail.

Missing evidence

Work cannot advance to ready; the owner, deadline, risk, and escalation are visible.

Conflicting sources

The discrepancy is flagged and the approved source hierarchy determines resolution.

Ownership gap

A task cannot remain unassigned; backup ownership and escalation activate.

Patient message conflict

Outdated content is retired and one source-controlled instruction is issued.

Automation failure

The error enters an exception queue, manual fallback begins, and no false completion is recorded.

AI-assisted output

Source, reviewer, edits, approval, uncertainty, and disposition remain traceable.

Downtime and recovery

Essential work continues safely and deferred transactions are reconciled after recovery.

> Pass condition: the future-state workflow makes missing evidence, uncertainty, exceptions, and ownership visible before closure.

CAREER POSITIONING

How this project supports Kori's 2026-2027 career.

Healthcare operations

Problem framing, workflow mapping, handoff design, exception ownership, KPIs, and closed-loop improvement.

Implementation support

Requirements, governance, UAT, issue tracking, training, adoption, change control, and sustainment.

Health informatics

Data definitions, source hierarchy, interoperability awareness, auditability, status design, and information quality.

Revenue cycle

Patient access, authorization, denial root cause, workflow dependencies, financial communication, and prevention.

Quality improvement

Current state, root-cause analysis, prioritization, countermeasures, testing, metrics, and feedback loops.

Consulting readiness

Structured analysis and professional deliverables without inflating experience or authority.

> **Recruiter language:** I developed a student-led, simulated, no-PHI healthcare consulting case study that translated a broad request for digital transformation into a structured operational engagement. I mapped the current state, separated symptoms from root causes, designed an implementation-readiness matrix, created synthetic UAT scenarios, and defined KPIs and a 30-60-90 day roadmap.

Source context: MarketsandMarkets healthcare consulting forecast (2025); CMS value-based care and CMS-0057-F interoperability/prior authorization resources; NIST AI Risk Management Framework; HHS healthcare cybersecurity performance goals; U.S. Bureau of Labor Statistics Management Analysts outlook.

DOWNLOADABLE PORTFOLIO PROOF

Review the complete branded case study.

The PDF contains the full simulated case, issue-tree logic, engagement lifecycle, implementation-readiness matrix, synthetic UAT, KPIs, career positioning, and student-level boundaries.